

Family Policy Report on the Compassionate Intervention Act in Alberta

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Executive Summary

To address the rise in drug toxicity deaths in Canada, especially opioid-related, provinces of the country are expanding state power to subdue individual autonomy by forcing substance users into involuntary treatment. In May 2025, Alberta passed its own special policy: the Compassionate Intervention Act. The program replaced a youth-only version of the program. Under this act, an authority figure may apply for the detention of a drug user, with or without their consent, to treatment. Youths (people aged 0-19) have a lower eligibility threshold.

Youths have a significant place in the opioid crisis, the purpose of the policy, and the public discourse of the policy. For youths, autonomy is not only controlled by the state, but also socially by their guardians. It is a question of the level of power guardians should have over children and the level of autonomy children should have independently, and how and how much the state should empower these parties. Youth autonomy is conflated with the argument that drug users cannot make their own decisions because of inebriation. Those for involuntary treatment have a paternalistic claim. Those against involuntary treatment see it as a violation to individual rights. Another claim by those against is that involuntary detention and the stigma of substance addiction breaks apart families. Though involuntary treatment lacks evidence, those for it frame the program as an experiment dubiously following international models.

Three policy options are presented to address the drug crisis that impacts youth, parents, and families: increase the spectrum of treatments, consult those directly served, and invest in and expand Compassionate Intervention. I recommend increasing the spectrum of treatment options (and abolishing involuntary treatment) to treat the diversity of needs and empower the positive impacts of autonomy.

Issue Overview

The opioid crisis in Canada is killing children at alarming rates. Future generations are lost and families are disrupted. The health of the individual is central to the well-being of the family. If fertility falters, the population that is living must remain living – especially the youth population. Frustration and trauma from failed responses has created different attitudes for how to address youth drug-related deaths, but they all want the same outcome: an urgent and effective response that will decrease and completely prevent the crisis.

For all age groups across Canada, the opioid crisis had a rough surge in the last 10 years. In 2019, 10.0 in 100,000 people in Canada died from apparent opioid toxicity according to the Public Health Agency of Canada (PHAC). This crude rate catapulted during the COVID-19 pandemic, to 17.0 in 2020 and a high of 21.0 in 2023 (see figure 1). Of all provinces and territories, British Columbia (BC) and Alberta suffered the most. From 2019 to 2023, BC rose from 20.2 to 47.4; Alberta rose from 14.4 to 39.9.

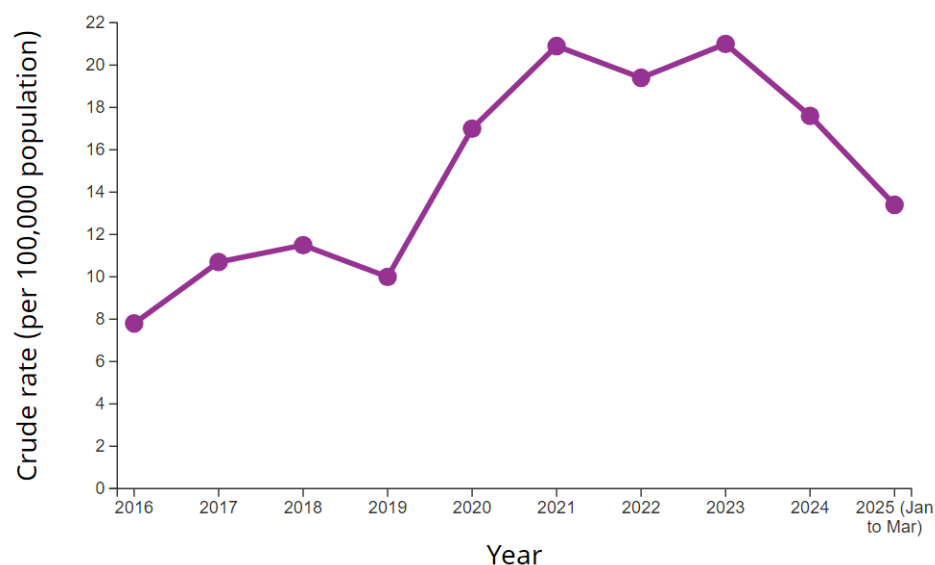


Figure 1 | Crude rate (per 100,000 population) of apparent opioid toxicity deaths in Canada from 2016 to 2025 (January-March) (PHAC 2025)

Alberta has unique policies specifically addressing the drug crisis and the youth drug crisis – though not specifically the opioid crisis. In 2006, the province passed the Protection of Children Abusing Drugs (PChAD) Act. It permitted the involuntary detention of substance-using youths under 18 into addiction treatment for a maximum of 15 days. Guardians were required to present evidence to a judge that the youth's drug use impacted their functioning in school, physical health, or another area (O'Brien and Hudson-Breen 2023). In 2023, Alberta introduced a new policy to replace the much criticized PChAD program – the Compassionate Intervention Act. The new act passed May 2025. The basis of the act is to allow “adult family members, guardians, healthcare professionals, peace or police officers” (Government of Alberta 2025) to submit an application to institutionalize anyone of any age into involuntary drug addiction treatment centers for up to 3 months or community-based care for up to 6 months.

Youth have a threshold of eligibility lower than adults “to allow an intervention before the point of imminent danger and life-threatening harm.” Youth receive age appropriate supports such as youth treatment centers, education, guardian involvement. This was billed as an improvement to PChAD for its longer term treatment, easier application process, and increased family involvement – common criticisms against the repealed program (Government of Alberta 2025).

The extremity of the program is attributed to the extremity of the crisis. In 2023, 1,867 Albertans died from opioid toxicity – this includes 56 youths (0-19 years of age). The number of deaths rose from ~12 youths in 2019 (out of 626 deaths). The proportion of youths that make up the death rate in Alberta has steadily been between 2-3% from 2016 to present year. This is higher than the national proportion steady rate of 1-2% (PHAC 2025).

The Government of Alberta, governed by the United Conservative Party since 2023 and led by Premier Danielle Smith, frames the Compassionate Intervention Act as the first of its kind. It marks this difference with the novel name: “involuntary (or forced or coerced) treatment” is now “compassionate intervention”. The new label for the same process

shows an awareness of the stigma behind “force” yet continued pursuit of it. The government frames involuntary treatment as a duty for the sake of those detained.

There is involuntary care for mental health disorders across Canada, but the programs don’t always include youth or substance abuse as a disorder. For example, in BC, consent is required for children ages 12-16 for them to receive substance abuse treatment. Since the act passed in Alberta, Saskatchewan also introduced a similar bill with the same name – the Compassionate Intervention Act (Government of Saskatchewan 2025).

The key stakeholders of the issue are: substance users and substance-using youths (0-18 years of age); their parents, guardians, and caregivers; the provincial government and associated bodies; treatment professionals such as doctors, therapists, caseworkers, social workers, and others; and advocacy groups.

Involuntary treatment of substance abuse and mental illness is criticized but not completely disavowed by each of these stakeholders. The escalation of this method by Alberta in drug use cases was met with heavy debate. Though the program included all ages, youths were a major focus of discussion. Further policy discussion can alter the bill, repeal it for another method, or affirm the adoption of the bill across Canada.

Claim Analysis & Fact-Checking

A) Deteriorated decision-making justifies loss of autonomy

A major claim of those in support of involuntary treatment is that substance users do not have the capacity to make good decisions, so outside agents must make that decision for them. State paternalism is evoked by the assertion – the Act evaluates the degree of power the state affords actors over the autonomy of others. In the context of family, this is the degree of parental/guardian power over the autonomy of a child.

Deciding the specific of who is given what power and over whom is important. There must be strong justification for action that is at risk of violating individual rights.

Proponents of involuntary treatment justify the terms of incapacity “logically”. There is some academic recognition that coercive treatment may be necessary to “restore autonomy” (CORE 2025:10). In a Government of Alberta information sheet on the Compassionate Intervention Act, they introduce the bill by explaining: “Addiction is an illness if left untreated can worsen and can eventually cause a person to lose their ability to make good decisions regarding their health and wellbeing.” No evidence is presented so it is framed as a natural conclusion that people who use substances can “lose their ability to make good decisions” (2025). The parameters of when that happens is left to the application process. The government asks for great trust in the system without setting formal limits in the policy. So any of the people permitted to submit an application against someone may be compelled by the vague criteria that is made applicable to anyone who uses drugs.

This claim is often made in the context of youth and their “powerless” parents, though Compassionate Intervention is for all ages. Patrick Brown, former leader of the Ontario Progressive Conservative Party from 2015 to 2018 and Mayor of Brampton from 2018 on, wrote an op-ed welcoming Compassionate Intervention into Ontario and calling for a pilot program in Brampton. Brown (2024) uses pathos by relaying the tragic story of a thirteen year old girl’s death from overdose to argue his case: “Her death is a heartbreaking reminder that for those too young or too ill to make rational decisions, the current system is failing.” Meanwhile, “her parents were powerless to intervene.” For news outlet Canadian Affairs, Alexandra Keeler reported on the death of a teenage girl and her father’s lawsuit against the BC government and several others. Their first-hand accounts justify the need for involuntary care. Brown and other advocates bear responsibility outside the individual to the rational actor. The framing extends into greater debates of parental control over children for their health and safety. The discourse of any related debate is evoked in the “powerless parent”. This garners support from non-primary stakeholders who are invested in the greater issue from other lenses. Bringing special emphasis to youth death is understandable – but as pointed

out earlier in this report, youth only make up 1-2% of opioid-related deaths in Canada. Youth do not represent the typical experience of drug users subject to the Act.

In opposition to this claim are those who uphold individual rights strictly. In a letter to Alberta's Minister of Public Safety and Emergency Services when Compassionate Intervention was first introduced in 2023, the representative group Moms Stop the Harm (MSTH) argues that the Mental Health Act, which permits involuntary treatment on the basis of mental health, is enough, insinuating the Compassionate Intervention Act over-stepped these boundaries (2023). MSTH uses local precedent to prevent expansion. Kelsall, Govorchin, and Patrick (2023) for The Conversation reference the categorization of involuntary treatment as inhumane in the Global South by international human rights organizations such as the United Nations Office on Drugs and Crime, UNAIDS, and Human Rights Watch. They use credible global precedent to denounce the practice, ethos rhetoric.

B) Families are broken apart by involuntary treatment

Proponents of involuntary care focus a great deal on the traditionally vulnerable population of youths. Another major concern in family sociology, however, is the capacity of adults to form and maintain families. If family is disrupted, then families are not maintained.

There are multiple ways family may be broken by involuntary treatment depending on circumstance:

1) Parents of youth in involuntary care

- Involuntary drug use treatment program PChAD traumatized children and families – including alienation from child and loss of child (MSTH's 2023, O'Brien and Hudson-Breen 2023).
- O'Brien and Hudson-Breen (2023) found that families lacked support from PChAD, detrimental to the health of caretakers.

2) Parents in involuntary care

- Tseris, Hart, and Franks (2022) interviewees shared while under involuntary care, they lost custody of children, visiting and caring conditions for children were weak.
- Cattapan and McKenzie (2025) frame involuntary care as creating climate of “punishment” (434) for expecting and current parents as custody is threatened and trust in healthcare providers is lost when they become agents of a punitive state.

3) Potential parents in involuntary care

- Tseris et al. (2022) gave a view into how motherhood plans were interrupted, delayed, and/or thwarted by involuntary care.
- In addition, an interviewee described how “paternalistic messages” (Tseris et al. 2022:656) from care staff discouraged pregnancy because of psychiatric diagnosis.
- Cattapan and McKenzie (2025) predicted that pregnant people will be targeted by involuntary care because of the cultural concern over fetal health in connection to substance abuse.

Tseris et al. (2022) are explicitly against involuntary treatment: “Concerningly, social work has largely demonstrated implicit support for involuntary treatment...” as they write (647). Cattapan and McKenzie (2025) are explicitly progressive in their feminist criticism of social constructions of motherhood and substance abuse. Their peer-reviewed academic journal articles are biased based on their belief.

The Tseris et al. (2022) study was exclusively made up of women who experienced involuntary mental health treatment in Australia. The different setting and focus on mental health rather than exclusively substance abuse weaken the study. The Tseris et al. (2022) study and O’Brien and Hudson-Breen (2023) study are both small, non-generalizable studies. Lacking quantitative studies of the experiences of parents and pregnant people.

C) Involuntary treatment lacks evidence

To justify the policy, the Government of Alberta directs to a white paper by the Canadian Centre of Recovery Excellence (CORE 2025). The systematic review outright concludes “there is scant high-quality research evidence to guide policy and practice for Alberta’s CI [Compassionate Intervention] initiative” (CORE 2025:3). According to CORE (2025), there are few comparable studies on the topic, there are no comparison studies with control groups, what exists is of low quality. Of note, only one study focused on youth was found. CORE ultimately frames Alberta’s Compassionate Intervention Act as an experiment that will inform other applications.

The Government of Alberta does not outright address the lack of evidence for involuntary treatment of substance use. Instead, the Government of Alberta, governed by the UCP and Premier Danielle Smith, frames the Compassionate Intervention Act as the first of its kind. They call it the “Made-in-Alberta strategy”, incorporating “the best elements of models from around the world”, though “no single jurisdiction mirrors Alberta’s proposed framework” (2025). This justifies the weak evidence presented in support of involuntary treatment. As Premier Danielle Smith elaborated: “You won’t know if something works until you try it” (Smith 2025). Even the allusion to using “the best elements of models from around the world” is misleading. European models in particular are referenced, especially Portugal’s. This is a frame bridging tactic, connecting the issues and solutions across contexts. However, as Alanna Smith (2023) found, the Portugal drug use intervention program that includes involuntary care is primarily built on decriminalization – not an avenue Alberta will be exploring. What is considered “the best element” is not based on evidence or consensus, but political opinion. The Government of Alberta can suggest the Compassionate Intervention method can be effective by bridging its frame to other systems. The previously mentioned improvements on PChAD advertised by the Government of Alberta were proposed in previous years by both governmental advisory reports (OCYA 2018) and researchers (O’Brien and Hudson-Breen 2023). In combination, a suggestion of evidence and a suggestion of improvement from the Government in its presentation of the Act

shows the importance of both in social policy. In particular, family policy is especially stirred by evidence and improvement because of the perceived vulnerability of the youth population.

Stakeholders against the Compassionate Intervention Act address a lack of evidence for involuntary intervention in a variety of ways. MSTH (2023) argue for total repeal of involuntary treatment and full dedication to evidence-based voluntary treatment. Kelsall et al. (2023) use academic evidence to prove the negative effects of involuntary treatment, such as increased risk of overdose and death, lack of impact on use after treatment, increased likelihood of re-admission. Before the introduction of Compassionate Intervention, the OCYA (2018) recommended an increase in a spectrum of treatments.

Policy Options

A) OCYA (2018) & O'Brien and Hudson-Breen (2023): Increase spectrum of treatments

On the PChAD act, OCYA reported: "Current strategies, in response to the opioid crisis, do not address the needs of children and youth" (2018:29). Rather than expand involuntary treatment, OCYA recommended an increase in the *variety* of interventions. They recommended: voluntary recovery with evidenced increased likelihood of success, specific services for youth opioid use needed, programs that address co-occurring issues (combination of issues in substance abuse and mental health and/or cognitive/intellectual disability). In addition, O'Brien and Hudson-Breen (2023) recommended those resistant to treatment not be forced into abstinence, but be supported by harm reduction services. They recommended: supervised consumption services, drug checking services, naloxone distribution.

Both parties are focused on drug-using youths at risk of involuntary treatment. OCYA's report is especially focused on opioid use. Both recommend state resources to be diverted from involuntary treatment to a spectrum of services they find to be more

effective. The goal is to respect the autonomy and diversity of youths using drugs, with a focus on equity. The root cause according to this policy perspective is a lack of programs that fit the different needs of people. It is viable if resources are freed by repealing involuntary treatment and redirecting those funds to a wider range of programs. Many of these programs are already in use across Canada and in Alberta, so the policy attempts to strengthen what works, not invent what could. The rights and autonomy of youth are affirmed while the paternal power of both state and guardian are undermined. This policy has the point of view that drug users are not overwhelmingly unwilling to get treatment but lack personalized resources.

B) OCYA (2018) & Tseris et al. (2022): Consult those directly served

As there are committees of health and academic professionals that advise policy, as well as public forums, OCYA (2018) & Tseris et al. (2022) recommend the inclusion of substance users, involuntary treatment survivors, and their supports (parents, caretakers, etc) in policy-making. OCYA calls to uplift youth drug users and their families, while Tseris et al. calls to center survivors of involuntary treatment (women in particular). Committees or forums for these individuals to be heard is how this may be implemented.

The goal of this option is to create and improve policy based on the lived experiences and needs of individuals who are most affected by state treatment. The focus is shifted from the impact on the public (regulation of substance users) to the impact on the most vulnerable (regulation of behaviours). The OCYA report affirms that, “[Youths] having ownership over their recovery increases their likelihood of success” (2018:35). Loss of autonomy is the root cause this policy recommendation is attempting to address. In addition, first-hand perspectives provide evidence missing from other sources like NGOs and academia. It is low-cost, though may be controversial politically as these are heavily marginalized groups. Indigenous, Black, and other racialized representation is particularly important for the disproportionate impact of the drug crisis and

institutionalization of these groups, as well as the need for culturally appropriate services that account for heritage traumas (Kelsall et al. 2023).

C) Government of Alberta (2025) & Brown (2024): Invest in and expand Compassionate Intervention

Mayor Patrick Brown suggested a Compassionate Intervention pilot program for Brampton in a bid to introduce the method to Ontario. The expansion of this policy across provinces provides more investigation in its effectiveness. The adoption of the method in multiple provinces means provincial and municipal resources may be used to explore and improve the method. The goal is to legitimize a method governmental bodies and political individuals believe to be effective to resolve an urgent crisis that requires immediate attention for its deadline. According to this policy recommendation, the root cause of opioid deaths is an over-reliance on individual responsibility to resolve addiction.

This expansion may struggle in economic feasibility as the program requires a big dedication of resources – Alberta was helped by its PChAD program, which already ran age-appropriate treatment centers for youth. The program has weak support from Brampton city councillors, so it is also politically unviable (Hamza 2024). There has been no news on Brown's proposal in more than a year since the announcement. Though compassionate intervention lacks evidence, this is masked by the novelty of the program. As Brown said in October 2024, "If [the Compassionate Intervention program] works it saves lives, if it doesn't we go back to the drawing board." (Rumbolt 2024). Brown, other politicians, and the media include issues of homelessness and mental illness to extend the framing to encompass popular social ills that may be fixed by this program as well. This is a bid to improve the look of the program as a fix-all, though lacking material conditions such as physical housing are a confounder. Desperation for a quick, novel fix colors this policy recommendation.

This policy values state and guardian paternalism. Accordingly, the family follows a more patriarchal, nuclear structure of subservient children and a head of family. The

right of children for autonomy may be safeguarded by protective legal procedure, but it is ultimately buried. Guardian power is asserted by the state; the state is extremely involved in maintaining family wellbeing.

Recommendation

I would recommend option A, increasing the spectrum of treatments. The research of OCYA (2018) & O'Brien and Hudson-Breen (2023) affirm evidence-based, voluntary treatment leads to long-term, more likely change. A diversity of treatments handles the diversity of needs of substance users – this label frames the group as a monolith. There is also a diversity in conditions, in race, indigeneity, age, gender, material conditions, family structure, disability, mental illness. For example, OCYA (2018) found an intersection of substance use and mental illness not addressed by PChAD – the Compassionate Intervention Act also fails to account for co-occurring conditions.

Drug users are aware of the dangers of drug use – with different levels of intervention available, non-abstinent harm reduction may be preferable. This still keeps them alive and as healthy as they could be – the opioid crisis is measured by deaths and hospitalizations, not degree of use. Youth drug users will benefit from this policy because rather than losing trust in authority as a result of force, they may build trust in certain services out of a sense of choice. Their guardians will benefit from relief out of immediate danger so that long-term plans may be arranged outside of involuntary treatment terms. Current and expecting parents who are drug users will benefit from voluntary choice in treatment that does not force children into state custody or discourage/target pregnant people. They continue having autonomy in not only their lives but the lives of their children. Overall, this policy supports the well-being of family as individuals and as a unit.

The biggest disadvantage to the policy is by far how resource-intensive it is. A high budget would be needed to implement, monitor, and supply multiple programs with different needs. While some parents oppose involuntary treatment (MSTH 2023), some

parents have indicated a desire for involuntary treatment and longer detentions (Keeler 2024; O'Brien and Hudson-Breen 2023). The greatest disadvantage to family is the non-normative empowerment of youth to choose treatment and what treatment, over the choices of their guardians or other authorities. This may cause social disruption and pushback from families. Per OCYA (2018), wider education on available and best-for-needs treatment is also needed, which may be part of the extended interventions.

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Source Evaluation Appendix

#	Source Title	Link	Author / Organization	Publication Date	Type of Source	Credibility (1-5)	Bias	Notes
1	Involuntary Treatment Legislation in Canada: Implications for Pregnant People and Parents	https://doi.org/10.3138/ijfab-2025-0005	Alana Cattapan, Holly A. McKenzie for International Journal of Feminist Approaches to Bioethics	22 November 2025	Journal Article	4	Left	About International Journal of Feminist Approaches to Bioethics: • Peer-reviewed • International, multi-discipline editorial board • Published by University of Toronto Press Stakeholders: • Authors/Organization: Researchers -> Tertiary (general interest) • Study subjects: Pregnant people and parents who are at risk of involuntary treatment for substance use -> Primary About bias: • Feminist article in feminist journal with a focus on ethics • Critical of negative social constructions of substance users & motherhood • Critical of involuntary treatment (IT) being used as implicit fetal rights law (laws that separate fetus and pregnant person) • Critical of IT for targeting youth and pregnant people • Critical of IT as paternalistic of state, prompts surveillance Framing: • Focus on involuntary treatment as "punishment" (434) -> Transformation • Critical of framing IT as "matter of public interest" and substance users as "inherently vulnerable" and "unable of providing informed consent" (437) • Critical of framing that "once the child is born, state intervention is not only legitimate but also justified" (435)

								Family sociology lens: • Rights of fetus, rights of pregnant people • Family formation Key Claims: • AGAINST involuntary treatment • Links concern over fetal health, presumptions of “good motherhood”, and substance abuse • Predicts pregnant people will be target of involuntary treatment because of this link • Reasons against: ○ Creates climate of punishment for parents/expecting – threat of custody loss & healthcare providers become agents of state ■ Custody loss is difficult to navigate legally and administratively ○ Supports fetal protection and control over pregnant people ■ Fetal protection infringes on woman’s rights & autonomy & health
2	Alberta Recovery Model: Compassionate Intervention	https://open.alberta.ca/publications/alberta-recovery-model-compassionate-intervention-path-to-recovery	Government of Alberta (United Conservative Party of Alberta)	15 April 2025	Government information sheet	3	Right	About Government of Alberta: • Right-wing party • United Conservative Party of Alberta elected as majority government in 2023, incumbent party from 2019 election • Premier: Danielle Smith • 2017 merger of Progressive Conservative Party and Wildrose Party Stakeholders: • Authors/Organization: Government -> Primary • Subjects: ○ Individuals who may be submitted to involuntary treatment for substance use -> Primary

								○ Individuals who may submit request for treatment: “adult family members, guardians, healthcare professionals, peace or police officers” -> Primary Framing: ● Frames addicted people as having lost ability for good decision-making -> Amplification + Individual Responsibility ● Paternalistic view ● Renaming “involuntary”, “forced”, “coercive” treatment to “compassionate intervention” -> Transformation + Moral ● Framed as first of its kind ○ “Made-in-Alberta strategy” ○ Section on the treatment being “evidence informed” with reference to policy white paper: “Alberta’s compassionate intervention model incorporates the best elements of models from around the world. No single jurisdiction mirrors Alberta’s proposed framework.” ● Within list of “key roles”, patients put in involuntary care are not listed – who matters are petitioners, public organizations and NGOs, and legal counsel ● Section on rights & review/appeal process, importance of safe-guards -> Rights ● Section on youth specifically and replacement of “Protection of Children Abusing Drugs Act” ● Section on “cultural connections” with particular focus on First Nations and Métis people – though integration is undefined Family sociology lens: ● Compassionate state intervention into family ● Preventative measures ● Health crisis response Key Claims: ● FOR involuntary treatment
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								• Safeguards in procedure to ensure appropriateness of measure & autonomy of treated individual • Variety in treatments – three months in treatment center or six months in community setting • Focus on aftercare: discharge plan ◦ May include help finding housing, employment, skills training • Specifies treatment that can not be refused: observation by treatment team, clinical advice, medication • Youth specific measures: ◦ “For youth, the threshold [of eligibility] would be slightly lower, to allow an intervention before the point of imminent danger and life-threatening harm” ◦ Age appropriate supports: separate centers, education, guardian involvement ◦ Improves on PChAD: long-term treatment, easier application process, increased family involvement • Cultural connection is important, may be integrated into intervention
3	B.C. parents powerless to help their addicted teens	https://www.canadianaffairs.news/2024/09/05/b-c-parents-powerless-to-help-their-addicted-teens/	Alexandra Keeler for Canadian Affairs	05 September 2024	News Article	3	Center	About Canadian Affairs: • Novel news site, founded 2023 • Private company owned by founder Lauren Heuser • Lauren Heuser: corporate lawyer, deputy section editor at the National Post (right-wing news) About journalist Alexandra Keeler: • Freelance reporter Stakeholders: • Author/Organization: Reporter -> Tertiary • Subjects: Substance users & parents of substance users -> Primary Framing: • Centers first-hand experiences

								• Pathos: detailed, real story of teenage girl who died from overdose, best friend who recovered and survived her, and father who is avenging his lost daughter • Parents of substance-using youths affirm lack of involuntary treatment hurt their children ◦ “It’s a B.C. law — you cannot force a minor into rehab without their permission,” said Sword. “You cannot parent your kid between the ages of 12 and 18 without their consent.” ◦ “I told the paramedic, ‘Our system is broken.’ And she just said, ‘Yes, I know.’” • Substance users give first-hand account of loss of capacity to consent ◦ “Mentally able to give consent?” said Grace. “No, I was never really mentally there.” • Uses legal action against governmental bodies & health agencies to legitimate claims • Problematizes “safe drugs” • Notes current B.C. system is “failed” Type of information: • Investigative, focused interviews Family sociology lens: • Guardian power over child autonomy • State empowering parental control Key Claims: • FOR involuntary intervention • Reasons for: ◦ Parents are powerless otherwise ◦ Youth who do not consent can’t be helped otherwise ◦ Unclear whether substance abusers have the capacity to consent to treatment ◦ Current BC law requires child ages 12-16 consent for treatment ■ Can only request involuntary treatment if physician or NP identifies mental disorder that needs treatment
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								• Safer supply program is harmful ○ Participants sell their prescribed safer supply ○ Lacking monitoring safeguards ○ Problem itizes safety of supply ○ Problem itizes existence of “safe supply” which makes users trust product ○ Improper doses, too many distributed
4	“Grasping at straws,” experiences of Canadian parents using involuntary stabilization for a youth’s substance use	https://doi.org/10.1016/j.drugpo.2023.104055	Daniel O’Brien, Rebecca Hudson-Breen for International Journal of Drug Policy	July 2023	Journal Article	4	None	About International Journal of Drug Policy: • Peer-reviewed • International, multi-discipline editorial board • Published by Elsevier Stakeholders: • Authors/Organization: Researchers -> Tertiary (general interest) • Study subjects: Parents who put children in involuntary care -> Primary Framing: • Presents both advantages and disadvantages of involuntary care • Study of “perception” of program by parents • Expands focus to include parental experiences -> Extension • Critical of “tough love” narrative that “punishment and confrontation is therapeutic” (7) • Critical of framing drug users as having character flaws that impact rationalization • Critical of common abstinence-only narrative, which facilitates belief long-term involuntary treatment is needed Type of information: • Qualitative study of interviews with 18 people • Not generalizable

								- Interviewees are parents of children who were put in Alberta PChAD (Protection of Children Using Drugs) program for involuntary treatment from 2007 to 2018 Family sociology lens: - Child and parental wellbeing – relationship maintenance, individual health to support family health - Family counseling - State intervention in family safety – caseworkers, hospitals, therapists, services Key Claims: AGAINST involuntary treatment (limited support) - Reasons for: - Protect youth, curb immediate danger, help those lacking capacity to help themselves - May be used as “leverage” to motivate change or as tool to frighten or punish - Reasons against: - Overrides autonomy, lack of evidence, may increase susceptibility to overdose, seems punitive - May lead to hesitancy to get help for physical help in fear of detainment - Instead some advocate for accessible voluntary treatment & harm reduction services - Loss of trust may result in hiding substance use - Parents with children put through involuntary treatment (PChAD) experienced... - Struggle to help youth - Joined program as last resort to curb overdose, violence, sexual exploitation, etc - Impact of drug use on school & recreational activities - Struggle to get support for selves - Confusion over how to be involved in child’s care in program - Difficulty in finding education, guidance, support
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								■ Responsibility to transition children to services after program placed on parents ○ Short term benefits with little lasting effects ■ Youth made to take break from drugs ■ Some thought reprieve was not worth risk of program ■ Many interviewees reported children returned to drug use ■ Want for longer detention ○ Anger, trauma, damaged relationships ○ Negative peer influence ● Proposed solutions: ○ According to experts, "...[involuntary] intervention should be reserved for use in exceptionally complex and high-risk situations, and only as one component of a comprehensive treatment approach" (7) ○ Improve access to voluntary treatment ○ For those who are resistant to treatment, prioritize harm reduction ■ Such as: supervised consumption services, drug checking services, naloxone distribution ○ Greater family support & reduce burden on family ■ Crisis response team ■ Evidence-based family counselling which doesn't need youth participation – improve well-being of caretakes
5	Open Letter to Mike Ellis, Alberta's Minister of Public Safety and Emergency Services	https://static1.squarespace.com/static/572aa55e3214012ea29505e/t/64527a7c1c40d56	Moms Stop the Harm	20 April 2023	Letter from NGO	3	Left	About Moms Stop the Harm: ● Organization representing Canadian families impacted by substance use-related harm and death Stakeholders: ● Organization: Representative -> Secondary ● Body of organization: Impacted families -> Primary ● Subject: Mike Ellis, Alberta Minister of Public Safety and Emergency Services -> Primary

		de5c7d87d/1758304035916/MSTH+letter+on+involuntary+care+-+final+final.pdf					About bias: • Opposes right-wing involuntary treatment and abstinence-only treatment • Advocates for prevention & harm reduction • Focus on “houselessness” and Indigenous & racialized experiences Framing: • Extension to issues of individual human rights, houselessness, disabled people, and Black, Indigenous, and other racialized people • Extension to advocate for other anti-drug initiatives: voluntary treatment, Narcotic Transition Service, civilian crisis response teams (to replace police) Type of information: • Letter of opposition with explanations and alternative demands Family sociology lens: • Degree of guardian control over child • Effect of trauma on family dynamic • Role of state service in maintaining family • Importance of health Key Claims: • AGAINST involuntary treatment • Reasons against: ◦ Violation of individual rights ◦ Treatment facilities lack regulation & don’t operate in evidence-based ways ◦ Doesn’t deal with long-term addiction treatment ◦ Traumatic experience with PChAD program – alienation from child or loss of child ◦ Voluntary care already flawed – long wait times, esp for opioid users, discharged into precarious housing conditions
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								○ Continues colonial violence – Indigenous and racialized people hospitalized, die, incarcerated in higher rates in relation to drug crisis ○ Evidence says it does not work and harms ● Proposed solutions: ○ Invest in voluntary treatment (evidence-based & accountable) ○ Repeal legislations against Narcotic Transition Service (service with alternative to toxic street drugs) ○ Replace police with civilian crisis response team ○ Exclude overdose from apprehension criteria ○ Eliminate involuntary treatment ○ Repeal policy that disproportionately targets Indigenous and racialized people ○ Repeal policy that targets drug users and disabled people
6	Compassionate intervention is the lifeline Canada's addiction crisis needs	https://www.ipolitics.ca/2024/10/10/compassionate-intervention-is-the-lifeline-canada-s-addiction-crisis-needs/	Patrick Brown for iPolitics	10 October 2024	Opinion article	3	Right	About Patrick Brown: ● Mayor of Brampton since 2018 ● Former leader of the Ontario Progressive Conservative Party from 2015 to 2018 ● Conservative, right-wing Stakeholders: ● Author: Mayor -> Primary ● Subjects: Substance users -> Primary Framing: ● Pathos: detailed, real story of teenage girl who passed away from an overdose ○ However, the Compassionate Intervention Act can detain anyone of any age by people who are not guardians -> Child tragedy used to affirm paternalism over all drug users ● Frames involuntary intercession as "compassionate intervention" ● Parents are "powerless to intervene" and addicted youth are "too young or too ill to make rational decisions"

								• Disenfranchises substance users as addiction “alters a person’s ability to make rational decisions” and “waiting for them to seek help is not realistic” • Ontario system framed as “failing” • Uses European systems (Portugal, Sweden) to legitimize -> Bridging • Extension to individual rights (and safeguards to protect them), stigma against addiction • Addiction not “moral failing” but “medical condition” -> Transformation Type of information: • Opinion news article Family sociology lens: • Evaluates degree of guardian power in autonomy of child • Intersection of child/family issues and substance use issues • Health of child to maintain family unit Key Claims: • FOR involuntary treatment • Involuntary treatment (IT) effect: ◦ Empowers parents ◦ Paternal guardianship over those who can’t decide for themselves ◦ Alleviates strain on law enforcement & healthcare ◦ Addresses disabling core of addiction • Gives importance to: ◦ Destigmatizing and not ignoring addiction ◦ New strategies, including involuntary treatment ◦ Human rights safeguards ◦ Need for urgency • Proposed solution: ◦ City of Brampton needs pilot project for compassionate intervention
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7	In B.C., Alberta and around the world, forcing drug users into treatment is a violent policy	https://theconversation.com/in-b-c-alberta-and-around-the-world-forcing-drug-users-into-treatment-is-a-violent-policy-205965	Tyson Singh Kelsall, Alya Govorchin, Lyana Patrick for The Conversation	25 May 2023	Opinion article	4	Left	About The Conversation: - Nonprofit news site, authors are academics adapting research into accessible news - Country of Origin: Australia About authors: - PhD student in Health Sciences, MSc candidate in Health Sciences, Assistant Professor of Indigenous Health - All from Simon Fraser University Stakeholders: - Authors: Researchers -> Tertiary - Subjects: Substance users -> Primary About bias: - Explicitly counters dehumanization of substance users & connects past colonial violence with present Framing: - Points to condemnations of involuntary treatment from human rights organizations such as the UN, UNAIDS, and HRW - Framing substance users as “people most affected by inequality and poverty” -> Transformation - Framing involuntary intervention as “locking people up against their will” Type of information: - Opinion news article - Evidence used: academic journal articles, news articles from PressProgress & CBC & Financial Post Family sociology lens:
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								• Policies impact adults and youth • Increase in forced treatment of youths Key Claims: • AGAINST involuntary treatment • Involuntary treatment (IT) effect: ◦ Increased risk of overdose & of death ◦ No impact on substance use post-treatment ◦ Deprives autonomy with no evidenced benefit ◦ Erodes trust in system ◦ Increased length of hospital stay, likelihood of readmission ◦ Increased suicidality ◦ Lower tolerance ◦ Continuation of settler colonial violence ■ BC Mental Health Act disproportionately targets Indigenous people • IT use has been increasing in BC because of 1) weak voluntary mental health services (long wait time, high fees, culturally incompatible) and 2) moral panic • Proposed solutions: ◦ Intervene in poisoned drug supply ◦ Compassion clubs • In 2020, BC NDP (governing party) proposed involuntary treatment for youth who overdosed but dropped the idea after pushback from IT survivors, drug policy experts, academics
8	"My Voice Was Discounted the Whole Way Through": A Gendered Analysis of Women's Experiences of Involuntary Mental Health Treatment	https://doi.org/10.1177/08861099221108714	Emma Jane Tseris, Eva Bright Hart, Scarlett Franks for Affilia: Feminist Inquiry in Social Work	November 2022	Journal article	4	Left	About Affilia: Feminist Inquiry in Social Work: • Peer-reviewed • US & Canada editorial board • Published by Sage Journals About authors: • Tseris is a senior lecturer in Social Work and Policy Studies

								• Hart & Franks are researchers & identified as survivors of involuntary treatment Stakeholders: • Authors/Organization: Researchers -> Tertiary • Authors Hart & Franks: Survivors of involuntary treatment -> Primary • Subjects: Women in involuntary mental health treatments -> Primary About bias: • Authors very cognizant of social constructions of “mental illness” (referred to as “distress”), “sanist assumptions” (biases against those designated as mentally ill), and the artificiality of “normality” • Intersectional feminist lens • Mindfully politicized analysis of “oppressive power relations and structural inequalities” (646) Framing: • Frames marginalized people put into involuntary treatment (IT) as “legitimate knowers” and experts of own experiences • Frames mental illness as social construction -> Transformation • Problemizes common narrative of IT as “violence as benevolence” where typically unacceptable, violent treated is justified as “care” • Problematizes common connection between mental illness and danger Type of information: • Qualitative interviews with 21 women • Not generalizable • Women over the age of 18 who experienced involuntary mental health treatment in Australia Family sociology lens: • Fear of loss of custody & child protection services involvement during treatment
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								• Bias discouraging motherhood • Treatment delaying motherhood • Associations of pregnancy and substance use with bad motherhood Key claims: • AGAINST involuntary treatment ◦ Explicitly: "Concerningly, social work has largely demonstrated implicit support for involuntary treatment..." (647) • Involuntary treatment effect: ◦ Delayed or thwarted plans for motherhood ◦ Children removed from care ◦ Fear of custody trouble disrupting authentic treatment ◦ Under-funded units for mothers & babies ◦ Loss of connection between mother and baby detrimental to baby health ◦ Visits from children not allowed ◦ "Paternalistic messages" (656) from care staff discouraging pregnancy because of psychiatric diagnosis • Proposed solutions: ◦ Use intersectional feminism, understand psychiatric coercion ◦ Acknowledge effect of medicalization and mental health unit trauma on distress ◦ Center survivors of system
9	Into Focus: Calling Attention to Youth in Opioid Use in Alberta	https://www.ocya.alberta.ca/wp-content/uploads/2024/06/InvRev_IntoFocus-OpioidReport_2018	The Office of the Child and Youth Advocate Alberta (OCYA)	June 2018	Government-associated report	5	Center	About the Office of the Child and Youth Advocate Alberta: • Independent office of the Alberta Legislature • Created under the provincial law, the Child and Youth Advocate Act • Advises government on youth rights and issues Stakeholders: • Organization: Representative -> Secondary • Subjects: Youth opioid users -> Primary • Audience: Government of Alberta -> Primary

		June_v2-1.pdf						About bias: • Non-partisan body Framing: • Pathos: detailed stories of 12 Albertan youths who passed away because of opioid poisoning • Generalization: "This crisis is far-reaching and is impacting children, youth and families from all backgrounds. There is no demographic that can identify who might be the victim of this tragic circumstance" (39). Type of information: • Investigative report • Evidence used: case studies, academic journal articles, expert opinion Family sociology lens: • About youth/children • About family maintenance by promoting health • Evaluates degree of guardian control in context of substance abuse Key claims: • NEUTRAL on involuntary treatment • "Current strategies, in response to the opioid crisis, do not address the needs of children and youth" (29). • On PChAD Act: "While this allows for abstinence and safety in the short term, it does not provide the necessary intervention and treatment that youth require" (8) • Proposed solutions: ◦ Health promotion – promote education among children with non fear-based information, to reduce stigma and remove barriers ◦ Intervene early – report on risk factors as informed by increased substance use intervention training for direct-service professionals
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								○ Increase spectrum of interventions – voluntary recovery increased likelihood of success, specific services for youth opioid use needed, address co-occurring issues ○ Increase family involvement – result in better outcomes, consider living environments ○ Re-evaluate Protection of Children Abusing Drugs Act – ■ Issues: ● lack of understanding from parents how it works because lack of information or assistance ● fractured relationships ● short stays didn't meet needs ● complex court process ● stigma ■ Recommendation: ● Make policy more reflective of needs of youth and their families ● Consult those served, youth and their families ● Barriers for youth include: ○ Acknowledgement substance use was problematic and help was needed in form of abstinence ○ Willingness to participate in recovery ○ Long waitlists ○ Did not fit service criteria ○ Lack of support for basic needs (i.e. housing) ○ Stigma ○ Lack of services that addressed co-occurring issues (concurrent addiction and mental health/cognitive disability issues) ● Barrier for parents/caregivers include: ○ Could not receive help from police, hospitals, community agencies because they didn't fit mandate ○ Stigma ○ Lack of information from service providers
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